4.9

PAEDIATRICS

Urology Infections

CHAPTER 4.9	UROLOGY INFECTIONS		
Infections and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
Acute cystitis *E. coli* *Proteus spp.*	**0-2 months old:** Ampicillin 25-50 mg/kg/dose IV **<7 days old:** q12H, **7-20 days old:** q8H, **≥ 21 days old:** q6H + Gentamicin 4-5 mg/kg/dose IV q24H		• Recurrent urinary tract infection due to underlying structural anomaly should refer to previous culture for choice of antibiotic.
	>2 months old: Cefuroxime 25 mg/kg/dose IV q8H	**>2 months old:** Cefuroxime 10-15 mg/kg/dose PO q12H **OR**	• Recurrent urinary tract infection due to underlying structural anomaly should refer to previous culture for choice of antibiotic. • *To check G6PD status prior to starting Co-trimoxazole.

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Infections and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
		Cefalexin 12.5- 25 mg/kg/dose PO q12H **OR** Co-trimoxazole 4-5 mg/kg/dose of Trimethoprim PO q12H* **OR** Nitrofurantoin 1-1.5 mg/kg/dose (max 100 mg) q6H **	• **Nitrofurantoin can only be used for patients >3 months old and is contraindicated in G6PD patients.
Acute pyelonephritis *E. coli* *Proteus spp.*	**0-2 months old:** Ampicillin 25-50 mg/kg/dose IV **<7 days old:** q12H, **7-20 days old:** q8H, **≥ 21 days old:** q6H + Cefotaxime 50 mg/kg/dose IV q8H for 10-14 days		• Repeat culture within 48 hours if poor response. • Antibiotics may be changed according to culture and sensitivity. • Suggest to continue IV antibiotic until afebrile 2-3 days then change to appropriate oral antibiotic after culture results.

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Infections and Likely Organisms	**Suggested Treatment**		**Notes**
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	>2 months old: Cefuroxime 25 mg/kg/dose IV q8H	**>2 months old:** Ceftriaxone 50 mg/kg/dose IV q12H	
Asymptomatic bacteriuria	No treatment is recommended.		
Urinary tract infection prophylaxis	Please refer to **Chapter 3.18: Non-surgical prophylaxis.**		